MEDICONNECT HOSPITAL MANAGEMENT SYSTEM

Deployment, Training & Handover Plan

Self-directed project simulation | Version 1.0 | 10 November 2026

Portfolio practice document — proposed approach only. No real hospital deployment is claimed.

1. Purpose

This plan describes the proposed activities required to prepare MediConnect for deployment, help hospital users adopt the system and transfer operational responsibility to the designated support team. It assumes that UAT, security checks, data migration validation and go-live approval have been completed. If any of these conditions is not met, deployment should be delayed or the decision formally reconsidered.

2. Deployment approach

A controlled, phased rollout is proposed rather than switching every department at once. The final rollout sequence should be agreed with hospital management after assessing department dependencies, patient volumes, staff readiness, infrastructure and support capacity.

Stage	Proposed activities
Preparation	Confirm approved release, validated data, user accounts, access permissions, backups, support contacts and rollback arrangements.
Pilot / limited release	If operationally suitable, start with a selected department or small user group using agreed safeguards and close monitoring.
Review pilot	Review incidents, user feedback, performance, data quality and support requests. Resolve critical issues before expanding.
Wider rollout	Expand to other approved departments in planned stages, with support available during each transition.
Stabilisation	Monitor performance, access, data integrity and user adoption; resolve early-life issues and communicate progress.

3. Pre-deployment readiness checklist

- The authorised sponsor / business owner has approved the go-live decision.
- UAT has been completed and acceptance evidence is recorded.
- Critical defects affecting patient safety, privacy, data integrity or core workflows are resolved.
- Patient data migration has been reconciled and validated by authorised personnel.
- User accounts, roles and permissions have been reviewed and tested.
- Backup, restore and recovery procedures have been tested and assigned to named technical owners.
- Primary and backup connectivity arrangements have been checked where applicable.
- A downtime procedure is documented, communicated and understood by relevant staff.
- Training materials, user guides and support contacts are ready.
- Deployment window, communications, technical coverage and escalation route are confirmed.
- A rollback or recovery decision process is agreed, including who can authorise it.

4. Training plan

Training should be role-based and use a safe training environment with test data. Staff should not practise with real patient records unless access and use have been specifically authorised under hospital policy.

User group	Training topics
Receptionists	Register patients, search for existing records, recognise duplicate warnings, manage appointments and follow the downtime process.
Doctors	Retrieve authorised records, add consultation notes, review patient history and understand record correction and audit requirements.
Nurses	Access permitted patient information, use approved clinical workflows and understand role restrictions and escalation.
Billing / Accounts	Create bills, review charge items, record payments, check balances and follow the correction process.
Administrators / Management	Manage authorised user functions, view dashboards and reports, and raise access or operational issues.
IT / Support	Manage approved support tasks, monitor system health, handle incidents, review logs and follow backup and recovery procedures.

5. Training delivery and completion evidence

- Schedule sessions around department shifts and service demands.
- Use demonstrations followed by supervised practice using test accounts and test data.
- Provide short role-specific guides and a clear route for questions or support.
- Record attendance and identify staff who need a repeat session.
- Use short task-based checks to confirm that users can complete essential workflows.
- Collect feedback and update guides where instructions are unclear.

6. Communications and support arrangements

When	Communication / support action
Before go-live	Tell affected departments what is changing, when it is changing, what they need to do and where to get help.
During rollout	Provide a named contact or help route, communicate known issues and send updates through the agreed hospital channel.
After rollout	Share progress, recurring issues, workarounds approved by the technical / business owners and any planned maintenance.
Escalation	Route clinical safety or patient-data concerns immediately to the designated clinical, privacy and technical authorities; escalate service outages to IT support and the Project Manager.

7. Handover to operations

Before the project team steps back, the following items should be handed to the designated operational owner:

- Approved system scope, configuration summary and release information.
- User and administrator guides, training materials and known limitations.
- Support contacts, incident categories, escalation route and service hours.

- Backup, restore, recovery and downtime procedures.
- Access-control and user-account administration process.
- Open defect list, accepted workarounds, known risks and planned follow-up actions.
- Maintenance arrangements, monitoring responsibilities and vendor / supplier contacts where applicable.
- Outstanding decisions, dependencies and items deferred to a later phase.

8. Roles and responsibilities

Role	Responsibility
Project Manager	Coordinates readiness decision, schedule, resources, escalations and formal handover approval.
Project Coordinator	Tracks checklist completion, arranges training and communications, records attendance, actions, decisions and handover evidence.
Technical Lead / IT Operations	Owns technical deployment, configuration, monitoring, backup/recovery and technical support handover.
Business Analyst / Product Owner	Confirms requirements and workflow understanding; supports user questions and records deferred requirements.
Department Champions	Support colleagues, gather feedback and direct issues through the agreed support route.
Hospital Sponsor / Authorised Owner	Approves operational readiness and accepts transfer of ownership on behalf of the hospital.

9. Handover acceptance record

Field	Record
Operational owner / department	_____
Handover date	_____
Training materials received	☐ Yes ☐ No ☐ Not applicable
Support and escalation route confirmed	☐ Yes ☐ No
Backup / recovery and downtime procedures received	☐ Yes ☐ No
Open defects / risks attached	☐ Yes ☐ No
Outstanding actions and owners	_____
Handover decision	☐ Accepted ☐ Accepted with actions ☐ Not accepted
Authorised operational owner approval	_____
Project Manager approval	_____

10. Project Coordinator's deployment checklist

- Confirm owners and due dates for every readiness action.
- Keep the readiness checklist and decision log up to date.
- Arrange user training and track attendance and follow-up needs.
- Send clear communications before, during and after rollout.
- Track incidents, defects, decisions and escalations during stabilisation.
- Confirm handover documents are received by the operational owner.
- Record formal acceptance and transfer any remaining actions with named owners.